

PACKAGE INSERT – MABRON RETARD

1. NAME OF THE MEDICINAL PRODUCT

MABRON Retard 100 mg prolonged-release tablets

MABRON Retard 150 mg prolonged-release tablets

2. QUALITATIVE AND QUANTITATIVE COMPOSITION

One prolonged-release tablet contains 100 mg tramadol hydrochloride.

One prolonged-release tablet contains 150 mg tramadol hydrochloride.

For the full list of excipients, see section 6.1.

3. PHARMACEUTICAL FORM

Prolonged-release tablet.

MABRON Retard 100 mg prolonged-release tablets are off white, round biconvex tablets, 9.1 mm diameter.

MABRON Retard 150 mg prolonged-release tablets are off white, capsule shaped tablets, 14.3 mm long.

4. CLINICAL PARTICULARS

4.1. Therapeutic indications

Treatment of moderate to severe pain.

4.2. Posology and method of administration

Posology

The dose should be adjusted to the severity of the pain and the individual clinical response of the patient. For doses not practicable with this medicinal product, other strengths of this medicinal product are available.

Unless otherwise prescribed, MABRON Retard prolonged-release tablets should be given as follows:

Adults and adolescents (12 years and older)

MABRON Retard is not approved for use in patients below 12 years old

The usual initial dose is 100mg twice daily, morning and evening.

Dependent upon the needs of the patient, subsequent doses may be administered earlier than 12 hours, but must not be administered earlier than 8 hours after the previous dose. **Under no circumstances should more than two doses be taken in any one 24 hour period.**

If the pain relief is insufficient, the dose may be increased to 150mg, twice daily or 200mg, twice daily.

MABRON Retard prolonged-release tablets should be swallowed completely, without breaking or chewing, independent of meals, with sufficient liquid.

The smallest effective analgesic dose should always be used. Daily doses of 400 mg of active substance should not be exceeded, except in special clinical circumstances.

Under no circumstances should MABRON retard be used for longer than absolutely necessary.

If long-term pain treatment with tramadol is necessary in view of the nature and severity of the illness, then careful and regular monitoring should be carried out (if necessary with breaks in treatment) to establish whether, and to what extent, further treatment is necessary.

Paediatric population

The safety and efficacy of MABRON has not been studied in the paediatric population. Therefore, use of MABRON Retard is not recommended in patients under 12 years of age.

Elderly

As a rule adjustment of the dose, in elderly patients (up to 75 years) without any clinical manifestations of hepatic or renal impairment, is not necessary.

In elderly patients (over 75 years) elimination may be prolonged. Therefore, if necessary the dosage interval is to be extended according to the patient's requirements.

Renal impairment, dialysis and hepatic impairment

In patients with serious renal or hepatic impairment the use of MABRON retard is not recommended. In moderate cases, an adjustment of the dosage interval may be considered.

Method of administration

Oral use

4.3. Contraindications

MABRON Retard prolonged-release tablets are contraindicated:

- in hypersensitivity to the active substance or to any of the excipients listed in section 6.1,
- in acute intoxication with alcohol, hypnotics, analgesics, opioids or psychotropic medicinal products,
- in patients receiving MAO inhibitors, or who have taken them within the last 14 days (see section 4.5),
- in patients with epilepsy not adequately controlled by treatment,
- for use in narcotic withdrawal treatment.
- children younger than 18 years to treat pain after surgery to remove the tonsils and/or adenoids
- adolescents between 12 and 18 years who are obese or have conditions such as obstructive sleep apnea or severe lung disease, which may increase the risk of serious breathing problems

4.4. Special warnings and precautions for use

MABRON Retard may only be used with particular caution in opioid-dependent patients, patients with head injury, shock, a reduced level of consciousness of uncertain origin, disorders of the respiratory centre or function, increased intracranial pressure.

In patients sensitive to opiates the product should only be used with caution.

Care should be taken when treating patients with respiratory depression, or if concomitant CNS depressant drugs are being administered (see section 4.5), or if the recommended dosage is significantly exceeded (see section 4.9) as the possibility of respiratory depression cannot be excluded in these situations.

Convulsions have been reported in patients receiving tramadol at the recommended dose levels. The risk may be increased when doses of tramadol exceed the recommended upper daily dose limit (400 mg).

In addition, tramadol may increase the seizure risk in patients taking other medicinal products that lowers the seizure threshold (see section 4.5). Patients with epilepsy or those susceptible to seizures should only be treated with tramadol if there are compelling circumstances.

Tolerance, psychic and physical dependence may develop, especially after long-term use. In patients with a tendency to drug abuse or dependence, treatment with MABRON Retard should only be carried out for short periods under strict medical supervision.

Tramadol is not suitable as a substitute in opioid-dependent patients. Although it is an opioid agonist, tramadol cannot suppress morphine withdrawal symptoms.

When a patient no longer requires therapy with tramadol, it may be advisable to taper the dose gradually to prevent symptoms of withdrawal.

Paediatric population

The safety and efficacy of MABRON has not been studied in the paediatric population. Therefore, use of MABRON is not recommended in patients under 12 years of age.

Respiratory depression

Administer MABRON cautiously in patients at risk for respiratory depression, including patients with substantially decreased respiratory reserve, hypoxia, hypercapnia, or pre-existing respiratory depression, as in these patients, even therapeutic doses of MABRON may decrease respiratory drive to the point of apnea. In these patients, alternative non-opioid analgesics should be considered. When large doses of tramadol are administered with anaesthetic medications or alcohol, respiratory depression may result. Respiratory depression should be treated as an overdose. Respiratory depression should be treated as an overdose. If naloxone is to be administered, use cautiously because it may precipitate seizures.

Cytochromes P450 (CYP) 2D6 Ultra-Rapid Metabolism

Some individuals may be CYP2D6 ultra rapid metabolisers. These individuals convert tramadol more rapidly than other people into its more potent opioid metabolites O-desmethytramadol (M1). This rapid conversion could result in higher than expected opioid-like side effects including life-threatening respiratory depression. The prevalence of this CYP2D6 phenotype varies widely and has been estimated at 0.5 to 1% Chinese, Japanese and Hispanics, 1 to 10% in Caucasians, 3% in African Americans, and 16-28% in North Africans, Ethiopians and Arabs. Data are not available for other ethnic groups.

Risks from Concomitant Use with Benzodiazepines

Profound sedation, respiratory depression, coma and death may result from the concomitant use of MABRON with benzodiazepines. Observational studies have demonstrated that concomitant use of opioids and benzodiazepines increases the risk of drug-related mortality compared to use of opioids alone. Because of these risks, reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate.

If the decision is made to newly prescribe a benzodiazepine and an opioid together, prescribe the lowest effective dosages and minimum durations of concomitant use.

If the decision is made to prescribe a benzodiazepine in a patient already receiving an opioid, prescribe a lower initial dose of the benzodiazepine than indicated in the absence of an opioid, and titrate based on clinical response.

Follow patients closely for signs and symptoms of respiratory depression and sedation. Advise both patients and caregivers about the risks of respiratory depression and sedation when MABRON is used with benzodiazepines. Advise patients not to drive or operate heavy machinery until the effects of concomitant use of the benzodiazepine have been determined. Screen patients for risk of substance use disorders, including opioid abuse and misuse, and warn them of the risk for overdose and death associated with the use of benzodiazepines (*See Drug Interactions*).

Serotonin Syndrome with Concomitant Use of Serotonergic Drugs

Cases of serotonin syndrome, a potentially life-threatening condition, have been reported during concurrent use of MABRON with serotonergic drugs (*See Interactions with Other Medicaments*). This may occur within the recommended dosage range.

Serotonin syndrome symptoms may include mental-status changes (e.g. agitation, hallucinations, coma), autonomic instability (e.g. tachycardia, labile blood pressure, hyperthermia), neuromuscular aberrations (e.g. hyperreflexia, incoordination) and/or gastrointestinal symptoms (e.g. nausea, vomiting, diarrhea) and can be fatal (*See Interactions with Other Medicaments*). The onset of symptoms generally occurs within several hours to a few days of concomitant use, but may occur later than that. Discontinue MABRON if serotonin syndrome is suspected.

Adrenal insufficiency

Cases of adrenal insufficiency have been reported with opioid use, more often following greater than one month of use. Presentation of adrenal insufficiency may include non-specific symptoms and signs including nausea, vomiting, decreased appetite, fatigue, weakness, dizziness and low blood pressure. If adrenal insufficiency is suspected, confirm the diagnosis with diagnostic testing as soon as possible. If adrenal insufficiency is diagnosed, treat with physiologic replacement dosing of corticosteroids. Wean the patient off of the opioid to allow adrenal function to recover and continue corticosteroid treatment until adrenal function recovers. Other opioids may be tried as some cases reported use of a different opioid without recurrence of adrenal insufficiency. The information available does not identify any particular opioids as being more likely to be associated with adrenal insufficiency.

Sexual Function/ Reproduction

Long term use of opioids may be associated with decreased sex hormone levels and symptoms such as low libido, erectile dysfunction, or infertility (*See Postmarketing Experience*)

4.5. Interaction with other medicinal products and other forms of interaction

MABRON Retard should not be combined with MAO inhibitors (see section 4.3).

In patients treated with MAO inhibitors in the 14 days prior to the use of the opioid pethidine, life threatening interactions on the central nervous system, respiratory and cardiovascular function have been observed. The same interactions with MABRON Retard as with MAO inhibitors cannot be ruled out during treatment with MABRON Retard.

Concomitant administration of MABRON Retard with other centrally depressant medicinal products, including alcohol may potentiate the CNS effects (See section 4.8).

The results of pharmacokinetic studies have so far shown that on the concomitant or previous administration of cimetidine (enzyme inhibitor) clinically relevant interactions are unlikely to occur. Simultaneous or previous administration of carbamazepine (enzyme inducer) may reduce the analgesic effect and shorten the duration of the action.

Benzodiazepines

Due additive pharmacologic effect, the concomitant use of opioids with benzodiazepines increases the risk of respiratory depression, profound sedation, coma and death.

The concomitant use of opioids and benzodiazepines increases the risk of respiratory depression because of actions at different receptor sites in the central nervous system that control respiration. Opioids interact primarily at μ -receptors, and benzodiazepines interact at GABA_A sites. When opioids and benzodiazepines are combined, the potential for benzodiazepines to significantly worsen opioid-related respiratory depression exists.

Reserve concomitant prescribing of these drugs for use in patients for whom alternative treatment options are inadequate (see Warnings & Precautions).

Limit dosage and duration of concomitant use of benzodiazepines and opioids, and follow patients closely for respiratory depression and sedation.

Serotonergic Drugs

The concomitant use of opioids with other drugs that affect the serotonergic neurotransmitter system has resulted in serotonin syndrome. If concomitant use is warranted, carefully observe the patient, particularly during treatment initiation and dose adjustment. Discontinue MABRON if serotonin syndrome reuptake inhibitors (SSRIs), serotonin and norepinephrine reuptake inhibitors (SNRIs), tricyclic antidepressants (TCAs), triptans, 5-HT₃ receptor antagonists, drugs that affect the serotonin neurotransmitter system (e.g. mirtazapine, trazodone, tramadol), monoamine oxidase (MAO) inhibitors (those intended to treat psychiatric disorders and also others, such as linezolid and intravenous methylene blue) (*See Warnings and Precautions*).

Caution should be exercised during concomitant treatment with tramadol and coumarin derivatives (e.g. warfarin) due to reports of increased INR with major bleeding and ecchymoses in some patients.

Other active substances known to inhibit CYP3A4, such as ketoconazole and erythromycin, might inhibit the metabolism of tramadol (N-demethylation) and probably also the metabolism of the active O-demethylated metabolite. The clinical importance of such an interaction has not been studied (See section 4.8).

In a limited number of studies the pre- or postoperative application of the antiemetic 5 – HT₃ antagonist ondansetron increased the requirement of tramadol in patients with postoperative pain.

4.6. Fertility, pregnancy and lactation

Pregnancy

Animal studies with tramadol revealed at very high doses effects on organ development, ossification and neonatal mortality.

Tramadol has been shown to cross the placenta. There are no adequate and well-controlled studies in pregnant women. Safe use in pregnancy use in pregnancy has not been established. MABRON is not recommended for pregnant women.

Tramadol – administered before or during birth – does not affect uterine contractility. In neonates it may induce changes in the respiratory rate which are usually not clinically relevant. Chronic use during pregnancy may lead to neonatal withdrawal symptoms.

Breast-feeding

Approximately 0.1% of the maternal dose of tramadol is excreted in breast milk. In the immediate post-partum period, for maternal oral daily dosage up to 400 mg, this corresponds to a mean amount of tramadol ingested by breast-fed infants of 3% of the maternal weight-adjusted dosage. For this reason tramadol should not be used during lactation or alternatively, breast-feeding should be discontinued during treatment with tramadol. Discontinuation of breast-feeding is generally not necessary following a single dose of tramadol.

4.7. Effects on ability to drive and use machines

Even when taken according to instructions, MABRON Retard may cause effects such as somnolence and dizziness and therefore may impair the reactions of drivers and machine operators. This applies particularly in conjunction with other psychotropic substances and alcohol.

4.8. Undesirable effects

The most commonly reported adverse reactions are nausea and dizziness, both occurring in more than 10% of patients.

Cardiovascular disorders:

Uncommon: effects on cardiovascular regulation (palpitation, tachycardia, postural hypotension or cardiovascular collapse). These adverse effects may occur especially on intravenous administration and in patients who are physically stressed.

Rare: bradycardia, increase in blood pressure.

Metabolism and nutrition disorders

Not known: hypoglycaemia

Nervous system disorders:

Very common: dizziness

Common: headache, somnolence

Rare: changes in appetite, paraesthesia, tremor, respiratory depression, epileptiform convulsions, involuntary muscle contractions, abnormal coordination, syncope.

Not known: speech disorders

If the recommended doses are considerably exceeded and other centrally depressant substances are administered concomitantly (see section 4.5) respiratory depression may occur.

Epileptiform convulsions occurred mainly after administration of high doses of tramadol or after concomitant treatment with drugs, which can lower the seizure threshold or themselves induce cerebral convulsions (see section 4.4 and section 4.5).

Psychiatric disorders:

Rare: hallucinations, confusion, anxiety, sleep disturbances and nightmares, delirium. Psychiatric adverse reactions may occur following administration of MABRON Retard which vary individually in intensity and nature (depending on personality and duration of treatment). These include changes in mood (usually elation, occasionally dysphoria), changes in activity (usually suppression, occasionally increase) and changes in cognitive and sensorial capacity (e.g. decision behaviour, perception disorders). Dependence may occur.

Eye disorders:

Rare: blurred vision, miosis

Not known: mydriasis

Respiratory disorders:

Rare: **respiratory depression**, dyspnoea

Worsening of asthma has been reported, though a causal relationship has not been established.

Gastrointestinal disorders:

Very common: nausea

Common: vomiting, constipation, dry mouth.

Uncommon: Retching, gastrointestinal irritation (a feeling of pressure in the stomach, bloating), diarrhoea.

Skin and subcutaneous disorders:

Common: sweating

Uncommon: dermal reactions (e.g. pruritus, rash, urticaria)

Musculoskeletal disorders:

Rare: motorial weakness

Hepato-biliary disorders:

In a few isolated cases an increase in liver enzyme values has been reported in a temporal connection with the therapeutic use of tramadol.

Renal and urinary system disorders:

Rare: micturition disorders (difficulty in passing urine, dysuria and urinary retention).

General disorders:

Common: fatigue.

Rare: Allergic reactions (e.g. dyspnoea, bronchospasm, wheezing, angioneurotic oedema) and anaphylaxis; Symptoms of withdrawal reactions, similar to those occurring during opiate withdrawal, may occur as follows: agitation, anxiety, nervousness, insomnia, hyperkinesia, tremor and gastrointestinal symptoms. Other symptoms that have very rarely been seen with tramadol discontinuation include: panic attacks, severe anxiety, hallucinations, paraesthesias, tinnitus, and unusual CNS symptoms (i.e. confusion, delusions, depersonalisation, derealisation, paranoia).

Postmarketing Experience:

Serotonin Experience:

Serotonin syndrome (*See Warnings & Precautions*)

Adrenal insufficiency (*See Warnings & Precautions*)

Androgen deficiency: Case of androgen deficiency have occurred with chronic use of opioids. Chronic use of opioids may influence the hypothalamic-pituitary-gonadal axis, leading to androgen deficiency that may manifest as low libido, impotence, erectile dysfunction, amenorrhea or infertility. The causal role of opioids in the clinical syndrome of hypogonadism is unknown because the various medical, physical, lifestyle and psychological stressors that may influence gonadal hormone levels have not been adequately controlled for in studies conducted to date. Patients presenting with symptoms of androgen deficiency should undergo laboratory evaluation.

Infertility: Chronic use of opioids may cause reduced fertility in females and males of reproductive potential. It is not whether these effects on fertility are reversible.

4.9. Overdose

Symptoms

In tramadol intoxication, in principle, the same symptoms occur as for all other central acting analgesics (opioids). In particular, these include miosis, vomiting, cardiovascular collapse, consciousness disorders up to coma, convulsions and respiratory depression up to respiratory arrest.

Treatment

The general emergency measures apply. Keep open the respiratory tract (aspiration), maintain respiration and circulation depending on the symptoms. The antidote for respiratory depression is naloxone. In animal experiments naloxone had no effect on convulsions. In such cases diazepam should be given intravenously.

In case of intoxication orally, gastrointestinal decontamination with activated charcoal or by gastric lavage is only recommended within 2 hours after tramadol intake. Gastrointestinal decontamination at a later time point may be useful in case of intoxication with exceptionally large quantities or prolonged-release formulations.

Tramadol is minimally eliminated from the serum by haemodialysis or haemofiltration. Therefore treatment of acute intoxication with MABRON Retard with haemodialysis or haemofiltration alone is not suitable for detoxification.

5. PHARMACOLOGICAL PROPERTIES

5.1. Pharmacodynamic properties

Pharmacotherapeutic group: other opioids; ATC code N02AX02.

Tramadol is a centrally acting opioid analgesic.

It is a non-selective, partial agonist of μ -, δ - and κ -opioid receptors with a higher affinity for μ -receptors. Other mechanisms contributing to the analgesic effect are the inhibition of the neural noradrenaline reuptake, and an enhancement of serotonin release.

Tramadol has an antitussive action. In contrast to morphine, analgesic doses of tramadol over a wide range have no respiratory depressant effect. Also gastrointestinal motility is less affected. Effects on the cardiovascular system tend to be slight. The potency of tramadol is reported to be 1 / 10 (one tenth) to 1 / 6 (one sixth) that of morphine.

Paediatric population

Effects of enteral and parenteral administration of tramadol have been investigated in clinical trials involving more than 2000 paediatric patients ranging in age from neonate to 17 years of age. The indications for pain treatment studied in those trials included pain after surgery (mainly abdominal), after surgical tooth extractions, due to fractures, burns and traumas as well as other painful conditions likely to require analgesic treatment for at least 7 days.

At single doses of up to 2 mg/kg or multiple doses of up to 8 mg/kg per day (to a maximum of 400 mg per day) efficacy of tramadol was found to be superior to placebo, and superior or equal to paracetamol, nalbuphine, pethidine or low dose morphine. The conducted trials confirmed the efficacy of tramadol. The safety profile of tramadol was similar in adult and paediatric patients older than 1 year (see section 4.2).

5.2. Pharmacokinetic properties

More than 90% of tramadol is absorbed after oral administration.

The mean absolute bioavailability is approximately 70%, irrespective of concomitant intake of food.

The difference between absorbed and non – metabolised available tramadol is probably due to low first-pass effect. The first-pass effect after oral administration is a maximum of 30%.

Tramadol has a high tissue affinity ($V_{d,\beta} = 203 \pm 40$ l). Protein binding is about 20%.

After administration of MABRON Retard 100 mg tablets the peak plasma concentration $C_{\max}$ 141 ± 40 ng / ml is reached after 4.9 hours. After administration of MABRON Retard 200 mg tablets a $C_{\max}$ 260 ± 62 ng / ml is reached after 4.8 hours.

Tramadol passes the blood – brain and placenta barrier. Very small amounts of the substance and its O – desmethyl derivative are found in the breast – milk (0.1 % and 0.02 % respectively of the applied dose).

Elimination of half-life $t_{1/2\beta}$ is approximately 6 h, irrespective of the mode of administration. In patients above 75 years of age it may be prolonged by a factor of approximately 1.4.

In humans tramadol is mainly metabolised by means of N – and O – demethylation and conjugation of the O – demethylation products with glucuronic acid. Only O – desmethyltramadol is pharmacologically active. There are considerable interindividual quantitative differences between the other metabolites. So far, eleven metabolites have been found in the urine. Animal experiments have shown that O – desmethyltramadol is more potent than the parent substance by the factor 2 – 4. Its half life $t_{1/2\beta}$ (6 healthy volunteers) is 7.9 h (range 5.4 – 9.6 h) and is approximately that of tramadol.

The inhibition of one or both types of the isoenzymes CYP3A4 and CYP2D6 involved in the biotransformation of tramadol, may affect the plasma concentration of tramadol or its active metabolite.

Tramadol and its metabolites are almost completely excreted via the kidneys. Cumulative urinary excretion is 90 % of the total radioactivity of the administered dose. In cases of impaired hepatic and renal function the half – life may be slightly prolonged. In patients with cirrhosis of the liver, elimination half – lives of 13.3 ± 4.9 h (tramadol) and 18.5 ± 9.4 h (O – desmethyltramadol), in an extreme case 22.3 h and 36 h respectively have been determined. In patients with renal insufficiency (creatinine clearance < 5 ml / min) the values were 11 ± 3.2 h and 16.9 ± 3 h, in an extreme case 19.5 h and 43.2 h, respectively.

Tramadol has a linear pharmacokinetic profile within the therapeutic dosage range.

The relationship between serum concentrations and the analgesic effect is dose – dependent, but varies considerably in isolated cases. A serum concentration of 100 – 300 ng / ml is usually effective.

Paediatric population

The pharmacokinetics of tramadol and O-desmethyltramadol after single-dose and multiple-dose oral administration to subjects aged 1 year to 16 years were found to be generally similar to those in adults when adjusting for dose by body weight, but with a higher between-subject variability in children aged 8 years and below.

In children below 1 year of age, the pharmacokinetics of tramadol and O-desmethyltramadol have been investigated, but have not been fully characterized. Information from studies including this age group indicates that the formation rate of O-desmethyltramadol via CYP2D6 increases continuously in neonates, and adult levels of CYP2D6 activity are assumed to be reached at about 1 year of age. In addition, immature glucuronidation systems and immature renal function may result in slow elimination and accumulation of O-desmethyltramadol in children under 1 year of age.

5.3. Preclinical safety data

On repeated oral and parenteral administration of tramadol during 6 to 26 weeks in rats and dogs and oral administration for 12 months in dogs haematological, clinico-chemical and histological investigations showed no evidence of any substance-related changes. Central nervous manifestations only occurred after high doses considerably above the therapeutic range: restlessness, salivation, convulsions, and reduced weight gain. Rats and dogs tolerated oral doses of 20 mg/kg and 10 mg/kg body weight respectively, and dogs rectal doses of 20 mg/kg body weight without any reactions.

In rats tramadol dosages from 50 mg/kg/day upwards caused toxic effects in dams and raised neonate mortality. In the offspring retardation occurred in the form of ossification disorders and delayed vaginal and eye opening. Male fertility was not affected. After higher doses (from 50 mg/kg/day upwards) females exhibited a reduced pregnancy rate. In rabbits there were toxic effects in dams from 125 mg/kg upwards and skeletal anomalies in the offspring.

In some in-vitro test systems there was evidence of mutagenic effects. In-vivo studies showed no such effects. According to knowledge gained so far, tramadol can be classified as non-mutagenic.

Studies on the tumorigenic potential of tramadol hydrochloride have been carried out in rats and mice. The study in rats showed no evidence of any substance-related increase in the incidence of tumours. In the study in mice there was an increased incidence of liver cell adenomas in male animals (a dose-dependent, non-significant increase from 15 mg/kg upwards) and an increase in pulmonary tumours in females of all dosage groups (significant, but not dose-dependent).

6. PHARMACEUTICAL PARTICULARS

6.1. List of excipients

Calcium hydrogen phosphate dihydrate (E341),
Hydroxypropylcellulose (E463),
Colloidal anhydrous silica (E551),
Magnesium stearate (E470b).

6.2. Incompatibilities

Not applicable.

6.3. Shelf life

3 years

6.4. Special precautions for storage

Store below 30°C in the original package.

6.5. Nature and contents of container

Al / clear PVC blisters in carton boxes in packs of 30 and 60 tablets.

Not all pack sizes may be marketed.

6.6. Special precautions for disposal

No special requirements.

7. MARKETING AUTHORISATION HOLDER

Komedic Sdn. Bhd., No. 4 Jalan PJS 11/14, Bandar Sunway, 46150 Petaling Jaya, Selangor, Malaysia

8. MANUFACTURER

Medochemie Ltd., (Factory AZ), 2, Michael Erakleous Street, Agios Athanassios Industrial Area, Agios Athanassios, Limassol, 4101, Cyprus

9. MARKETING AUTHORISATION NUMBER(S)

MABRON Retard 100mg: MAL09062122AZ

MABRON Retard 150mg: MAL09062123AZ

10. DATE OF FIRST AUTHORISATION/RENEWAL OF THE AUTHORISATION

MABRON Retard 100mg: 25/06/2009

MABRON Retard 150mg: 25/06/2009

11. DATE OF REVISION OF THE TEXT

February 2020